ISSUED, VALID FROM 30 SEPTEMBER 2026

Meningococcal ACWY (Travel and Hajj/Umrah)

Patient Group Direction, version 009, issued 30 September 2026. Three products: Nimenrix, MenQuadfi and Menveo.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Scope of this PGD

This PGD authorises the administration of a quadrivalent meningococcal ACWY conjugate vaccine for travel-related protection, including the certificate required for entry to Saudi Arabia for Hajj and Umrah.

- **The three products have different licensed minimum ages, different presentations and different schedules. Choose the product first, then follow that product's schedule. Do not mix schedules between products.**
- Where a course has been started with one product, complete it with the same product wherever possible.
- **Observe every patient for 15 minutes after vaccination.**
- This PGD does not cover outbreak or contact management, which is directed by the local UKHSA Health Protection Team. Refer.
- This PGD does not permit supply of vaccine to the patient for administration elsewhere.

Meningococcal ACWY conjugate vaccine

Summary of Green Book chapter 22 guidance for meningococcal ACWY

UKHSA, Immunisation Against Infectious Disease, chapter 22, Meningococcal. Chapter dated 10 June 2025; GOV.UK page last updated 30 July 2025. Summarised 9 September 2026.

Overview

Invasive meningococcal disease is caused by Neisseria meningitidis. Of the 12 capsular groups, B, C, W and Y cause most UK disease.

Spread is by aerosol, droplets or direct contact with respiratory secretions of a carrier, and usually needs frequent or prolonged close contact. There is marked seasonality, peaking in winter.

Incubation is two to seven days. Onset ranges from a mild prodrome to fulminant illness with death within 24 hours of the first symptoms.

The infection is fatal in 5 to 10 per cent of cases. Survivors may develop hearing loss, severe visual impairment, communication problems, limb amputation, seizures and brain damage.

Incidence is highest in infants under one, with a secondary peak at 15 to 19 years. Risk factors include smoking, preceding viral infection and living in closed or semi-closed communities such as university halls or barracks.

Who the chapter recommends it for

ROUTINE: one dose of MenACWY conjugate vaccine at around 14 years. Anyone who missed it should be offered a dose UP TO THEIR 25TH BIRTHDAY, including people new to the UK.

TRAVEL: every traveller should have a careful risk assessment covering itinerary, duration and planned activities. Those particularly at risk are visitors who live or travel rough, such as backpackers, and those living or working with local people.

HAJJ AND UMRAH: proof of vaccination against groups A, C, W and Y is a VISA ENTRY REQUIREMENT for pilgrims and seasonal workers travelling to Saudi Arabia. It is also recommended for Umrah at any time of year.

SUB-SAHARAN AFRICA: epidemics occur unpredictably across tropical Africa, particularly in the savannah in the dry season from December to June. Vaccination is recommended for long-stay or high-risk visitors, for example those living or working closely with local people, or backpacking.

Travellers should have a quadrivalent ACWY vaccine EVEN IF they previously had MenC conjugate vaccine.

OCCUPATIONAL: laboratory staff handling N. meningitidis must have MenACWY and 4CMenB, with boosters of both every five years.

MEDICAL RISK GROUPS: asplenia, splenic dysfunction, and complement disorders including those on complement-inhibitor therapy. Those starting a complement inhibitor should be vaccinated at least two weeks beforehand, or given prophylactic antibiotics until two weeks after vaccination.

Schedule and products

Licensed ages differ by product: Nimenrix from 6 weeks, MenQuadfi from 12 months, Menveo from 2 years. Check which product is in hand against the patient age.

From one year of age, including adults: a single 0.5 mL dose.

Under one year: the Green Book gives two doses four weeks apart. This PGD follows the licensed Nimenrix schedule in part 3, which differs; the Green Book interval is not used here.

BOOSTERS: the chapter states that booster doses in at-risk individuals are currently NOT recommended, because the need for and timing of boosters has not yet been determined. The five-yearly booster applies specifically to the occupational laboratory group.

Give intramuscularly into the deltoid, or the anterolateral thigh in infants of one year and under. May be given with other vaccines at a separate site, preferably a different limb, or at least 2.5 cm apart.

Menveo and Nimenrix are powder plus solvent: after reconstitution the entire 0.5 mL is drawn up and used immediately. MenQuadfi is supplied as a solution.

Contraindications and precautions

Do not give to anyone who has had a confirmed anaphylactic reaction to a previous dose, or to any component or manufacturing residue.

There are very few individuals who cannot receive meningococcal vaccines. Where in doubt, seek specialist advice rather than withholding.

Minor illness without fever or systemic upset is not a reason to postpone. If acutely unwell, postpone until recovered.

Previous systemic or local reaction is not a bar to further doses, on the same terms as the other chapters.

Pregnancy and immunosuppression

PREGNANCY: meningococcal vaccines may be given when clinically indicated. There is no evidence of risk from inactivated vaccines in pregnancy or breastfeeding, and where immunisation has been given inadvertently in pregnancy there has been no evidence of harm to the foetus.

IMMUNOSUPPRESSION: give in accordance with the routine schedule, including in HIV regardless of CD4 count. A full antibody response may not be made; consider re-immunisation after treatment finishes.

A supply point for a private service

The chapter states that vaccines for private prescriptions, occupational health use or travel are NOT provided free of charge and must be ordered from the manufacturers.

Patient Group Direction for the administration of Nimenrix, MenQuadfi or Menveo for travel-related protection against invasive meningococcal disease caused by groups A, C, W and Y.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these vaccines is a controlled drug, so registered pharmacy technicians may lawfully supply and administer under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- All users must be trained to the National Minimum Standards and Core Curriculum for Immunisation Training, and be competent in intramuscular vaccination technique.
- All users must be competent in the recognition and immediate management of anaphylaxis, and hold current basic life support training.
- All users must be able to distinguish between the three licensed MenACWY products, whose licensed age ranges, presentations and reconstitution differ. Choose the product first, then follow that product's schedule.
- All users must understand the Saudi Arabian certificate requirements for Hajj and Umrah, including the requirement that the certificate states the vaccine given was a conjugate vaccine.
- All users must be competent in assessing consent in children and young people, including parental responsibility and Gillick competence.
- All users must be competent in vaccine handling and cold chain management.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Active immunisation against invasive meningococcal disease caused by Neisseria meningitidis groups A, C, W and Y, for travellers, for pilgrims travelling to Saudi Arabia for Hajj or Umrah, and for those at occupational or lifestyle risk, in accordance with Green Book chapter 22.
Inclusion criteria	● Aged 6 weeks and over for Nimenrix; 12 months and over for MenQuadfi; 2 years and over for Menveo. There is no upper age limit for any of the three. ● Travelling to an area where MenACWY is recommended, travelling to Saudi Arabia for Hajj or Umrah, or otherwise at increased risk as described in Green Book chapter 22. ● **Valid informed consent obtained. Where the patient is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis of the assessment recorded.** ● No exclusion criterion present.
Exclusion criteria	**Refer, do not vaccinate, where any of the following applies.** ● **Age below the licensed minimum for the product held. Nimenrix is the only product licensed below 12 months, and Menveo cannot be used below 2 years.** ● Confirmed anaphylactic reaction to a previous dose of the same vaccine, or to any of its excipients or manufacturing residues. ● For Menveo specifically: hypersensitivity to diphtheria toxoid or

	CRM197, which is its conjugate carrier. Nimenrix and MenQuadfi are conjugated to tetanus toxoid instead, so this exclusion is product specific. ● Acute severe febrile illness. Postpone until recovered. A minor illness without fever is not a reason to defer. ● Outbreak or contact management, which is directed by the local UKHSA Health Protection Team and is outside a private travel PGD. ● A MenACWY conjugate vaccine dose within the last 5 years, other than the 12 month booster of the infant Nimenrix schedule. A repeat dose is authorised only as set out under Repeat doses (previous dose more than 5 years ago and a certificate required); an at-risk individual who may need an earlier booster is referred. ● Under 16 with no person with parental responsibility available to consent, and not assessed as Gillick competent.
Cautions	● Pregnancy. The Green Book position is that meningococcal vaccines may be given in pregnancy when clinically indicated, and there is no evidence of harm from inadvertent vaccination. ● Immunosuppression, including HIV regardless of CD4 count: vaccinate in accordance with the routine schedule, but the individual may not make a full antibody response. ● Bleeding disorders or anticoagulation: use a fine needle (23 gauge or finer) and apply firm pressure without rubbing for at least 2 minutes. ● Individuals due to start a complement inhibitor such as eculizumab, and those with asplenia or complement deficiency, may be eligible for NHS-funded vaccination. Check before charging privately. ● A previous MenC dose does not shorten or alter the schedule. ● Syncope can occur before or after vaccination, particularly in adolescents. Vaccinate seated and observe.
Actions if the patient is excluded or declines	Explain why vaccination cannot be given today and what the alternative is. Where the exclusion is age, name the product that would be suitable, or refer. Where it is acute febrile illness, arrange to vaccinate after recovery and, if travel is imminent, say plainly that protection may not be achieved in time. Where outbreak or contact management is involved, refer to the GP or the UKHSA Health Protection Team. Document the advice and the decision.

The medicine

Products, presentations and licensed ages	Nimenrix (powder and solvent for solution for injection). Licensed from 6 weeks of age. MenQuadfi (0.5 mL solution for injection, single-dose vial or syringe). Licensed from 12 months of age. Menveo (powder and solvent for solution for injection). Licensed from 2 years of age. **Confirm which presentation is held before use. Nimenrix and Menveo require reconstitution; MenQuadfi does not.**
Legal category	POM.
Route and method	Intramuscular injection, 0.5 mL whichever product is used. Anterolateral thigh in infants under 1 year of age; deltoid from 1 year of age and in adults. Do not give intravascularly, subcutaneously or intradermally.
Dose and frequency	**0.5 mL, whichever product is used. The number of doses depends on age and product.** NIMENRIX, infants 6 weeks to under 6 months: two doses at least 2 months apart, with a booster at 12 months of age if the primary course was completed

	before 12 months. **NIMENRIX, infants 6 to 11 months: a single dose, with a booster at 12 months of age at least 2 months after the primary dose (SmPC).** ALL PRODUCTS, from 12 months of age and in adults: a single dose.
Repeat doses	Routine boosters are not recommended for most travellers. **A repeat dose IS authorised under this PGD where the previous dose was given more than 5 years ago and a valid certificate is required for travel to Saudi Arabia. Record the date of the previous dose and the reason for the repeat.** JCVI has not determined the need for or timing of boosters in at-risk groups such as asplenia or complement deficiency. Do not invent an interval for those groups; assess individually and refer where there is doubt.
Quantity	One dose per patient per attendance. This PGD does not permit supply of vaccine to the patient for administration elsewhere.
Adverse effects	Very common: injection site pain, redness and swelling; irritability, drowsiness and loss of appetite in infants; headache and fatigue in older children and adults. Common: fever, nausea, malaise. Uncommon: rash, injection site induration lasting more than 3 days. Rare: anaphylaxis. See the anaphylaxis provision below.
Anaphylaxis and observation	**Adrenaline (epinephrine) 1 in 1,000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance and a telephone.** All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life support training. **Observe every patient for 15 minutes after vaccination. Vaccinate seated.**
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	● That valid informed consent was given, and from whom. Where the patient is a child, record the name and relationship of the person with parental responsibility, or the basis of the Gillick competence assessment. ● Patient name, address, date of birth, and the GP with whom they are registered. ● Name and brand of vaccine, batch number and expiry date. ● Date of administration, dose, route and anatomical site used. ● Which product was given and, where a course is involved, the dose number and the date the next dose is due. ● Where a repeat dose is given for certificate purposes, the date of the previous dose and the reason for the repeat. ● Name and registration number of the healthcare professional administering. ● Advice given, including advice given if the patient is excluded or declines. ● Details of any adverse drug reactions and the actions taken. ● That the vaccine was administered under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday, or the 26th where the individual was 17 when the

	dose was given.
Storage and cold chain	Store at +2C to +8C in the original packaging to protect from light. Do not freeze; discard if frozen. Reconstitute immediately before use. In-use stability after reconstitution has been demonstrated for 8 hours, but delay is not recommended. **COLD CHAIN EXCURSION: vaccine exposed to conditions outside the stated range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use. Do not administer excursion stock until that assessment is complete.**
Disposal	Dispose of used syringes, needles and any reconstituted vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Co-administration	May be given at the same time as other vaccines. Use a separate limb where possible, or sites at least 2.5 cm apart in the same limb. Record the site used for each vaccine.

Information for the patient

Written information	Supply the patient information leaflet for the product given. Where a certificate is required, supply it before the patient leaves and check the details are correct.
Counselling	● **For Hajj or Umrah the dose must be given at least 10 days before arrival in Saudi Arabia. Book accordingly.** ● **Check your certificate before you travel. It must state that the vaccine you were given was a CONJUGATE vaccine. If the type is not stated, the Saudi authorities treat the certificate as valid for 3 years only, regardless of what was actually given.** ● This vaccine protects against groups A, C, W and Y. It does not protect against group B meningococcal disease, which is a separate vaccine. ● **Know the signs of meningitis and septicaemia and seek help immediately whatever your vaccination status: fever, severe headache, neck stiffness, dislike of bright light, a rash that does not fade under pressure, cold hands and feet, drowsiness or confusion.** ● Some soreness, redness and mild fever are common in the first day or two and settle without treatment. ● Where a further dose is due, we have booked it. Attend for it; a single dose in an infant course is not a complete course.
Follow-up	Where a course is involved, book the next dose at this appointment. Seek medical advice urgently at any time for the meningitis and septicaemia symptoms listed above. For a routine query about the vaccine or the certificate, contact the pharmacy.

Appendix 1 , Schedules and certification

Nimenrix, 6 weeks to under 6 months	• Two doses at least 2 months apart. • Booster at 12 months of age if the primary course was completed before 12 months.
Nimenrix, 6 to 11 months	• A single dose, with a booster at 12 months of age at least 2 months after it. • That booster is the second and final dose of the infant course, given in the second year of life.
All products, 12 months and over	• A single dose. • MenQuadfi from 12 months. Menveo from 2 years. Nimenrix at any age from 6 weeks.
Repeat for certificate	• Authorised where the previous dose was more than 5 years ago and a valid certificate is required. • Record the previous dose date and the reason.

Certification for Hajj and Umrah

- Saudi Arabia requires proof of MenACWY vaccination for entry for Hajj, and for Umrah at any time of year.
- A conjugate vaccine given within the last 5 years is accepted. A polysaccharide vaccine is accepted within the last 3 years.
- **In both cases the dose must have been given at least 10 days before arrival.**
- **The certificate must state that the vaccine given was a conjugate vaccine. If the type is not stated, the certificate is treated as valid for 3 years only, whatever was actually given.**
- All three products in this PGD are conjugate vaccines. Say so on the certificate.

Saudi entry requirements are reviewed annually. Check the current requirements on TravelHealthPro before relying on the intervals above.

Authorisation

Version	v009
Supersedes	Version 008, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Change history to version 003

Later versions are recorded in the Version and change record at the end of this document.

Version	v003
Date	7 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● Full clinical review and reissue.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
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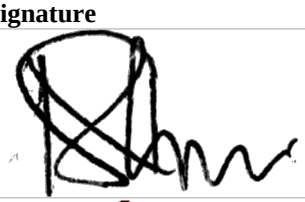
Version and change record
Version: v009
Issued: 30 September 2026
Supersedes: Version 008, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.
This PGD's staff section did not carry the requirement, common to every other Get Real Health PGD, that users have completed training in working under Patient Group Directions and been assessed as competent to work under a PGD before first working under this one. It is added, with the training-before-first-use statement.

Previous version records
Version: v008
Issued: 14 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
A dose within the last 5 years is now listed under the exclusion criteria, which is what the repeat doses row already implied; the repeat for a certificate after 5 years is unchanged.
The Appendix 1 row for infants aged 6 to 11 months now says that the 12 month booster is the second and final dose of the infant course, rather than reading as a separate dose.
The retention row now carries the 26th birthday clause used in the other PGDs.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v004
Issued: 10 September 2026

Supersedes: Version 003, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.
Version: v005
Issued: 10 September 2026
Supersedes: Version 004, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Infants under one year: the guidance summary gave the Green Book traveller schedule (two doses four weeks apart) and the PGD gave two doses two months apart; the 6 to 11 month row matched neither source. The PGD follows the licensed Nimenrix schedule: 6 weeks to under 6 months, two doses 2 months apart with a booster at 12 months; 6 to 11 months, a single dose with a booster at 12 months at least 2 months later. The summary now says the Green Book interval is not used.

Signed on behalf of Get Real Health

Version v009, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.